

CORRESPONDENCE BUNDLE — PAY AND THE EXCLUSION

Cory Shepherd (388372), AO3, Switchboard Services, Logan Hospital · original emails, payslip and messages, 2 July – 31 August 2026, in date order · compiled 2 September 2026

Each tab is the original document as sent or received. Nothing has been retyped. The two-page brief "Pay and the exclusion — the record" summarises this bundle and cross-refers to it.

Tab	Date	From / to	What it is	Pages
1	2 – 3 Jul 2026	Taylor ↔ Shepherd	ECC / leave type — the origin: exclusion pending ECC; sick-leave default; leave type confirmed as ANNUAL at 3:18 pm 3 Jul (4 messages)	2–5
2	7 – 13 Jul 2026	Forrest / Harrison ↔ Shepherd	ECC further information — paid-time request 10 Jul; QSuper deflection 13 Jul 3:53 pm; "I ask again" + interim duties offer 13 Jul 4:39 pm (4 messages)	6–17
3	15 Jul 2026	Harrison → Shepherd (and Zappia)	Pay refused — "sits with your QSuper Claim Manager"; interim duties refused "in any capacity"; the discretion paragraph	18–25
4	22 Jul 2026	Shepherd → Payroll	Payroll Enquiry 4438861 — "Pending Investigation"	26
5	28 Jul 2026, 5:39 pm	Shepherd → Injury Management (cc union, fund)	Incorrect application of EB12, Award and QH policies — contests the sick-leave-no-pay coding and the QSuper basis	27–32
6	29 Jul 2026, 4:10 pm	MSH → Shepherd	Reply to the 28 July letter	33–41
7	30 Jul 2026, 11:25 am	Shepherd → MSH	Reply — roster, pay and leave "remain live in parallel" (with attachments)	42–59
8	30 Jul 2026, 2:33 pm	MSH → Shepherd	Holding reply — delegate approval (with attachments)	60–80
9	5 Aug 2026, 7:30 am	Shepherd → Taylor / IM / HR	Stage 1 reply — cover email (pay restoration and leave re-credit sought)	81–85
10	12 Aug 2026	Payslip	Pay advice, fortnight 20 Jul – 2 Aug 2026: "NP Sick Leave" 76.00 hours, gross \$0.00	86
11	13 Aug 2026, 3:48 pm	Harrison → Shepherd	Reply — consent and EAF	87–92
12	18 – 28 Aug 2026	Roberts ↔ Shepherd (cc Petering, McQuillan)	"Follow up on Enquiries" — meeting arranged; 24 Aug: leave and AVAC requested, authority provided; 28 Aug: Stage 2 referral withdrawn, AVAC update sought, psychiatrist unanswered	93–104
13	Aug 2026	SMS — Roberts ↔ Shepherd	Text thread: the move, "hand my keys over", partner attended consults, prior work history	105
14	25 Aug 2026, 3:08 pm	Roberts → Shepherd, McQuillan, Petering	Teams invitation to continue the 21 Aug meeting	106–108
15	31 Aug 2026	Shepherd ↔ Taylor (cc Roberts, Petering)	10:59 application (LSL from 13 Jul, A/L on exhaustion, AVAC today, under protest, re-credit) → 3:34 pm Taylor: one fortnight, PRN 249 863 66, "0.5 FTE", LSL blocked → 4:06 pm four-point correction	109–115

Total: 115 pages including this index. Prepared for Together Queensland. Employment matter only.

Re: Cory - ECC/Leave Type

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au

Sent: Friday 3 July at 15:18

Hi Chloe,

Thanks for that. As the time off was due to my medical condition and my paid sick leave is exhausted, could you please process the balance as annual leave rather than sick leave without pay.

On returning the completed Employee Capability Checklist went to the Injury Management team today. It certifies me fit to resume from 3 July 2026. Could you please confirm I'm right to attend, or otherwise let me know my next returning shift.

Kind regards,

Cory

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Friday, July 3, 2026 2:49:32 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Cory Shepherd

<Cory.Shepherd@health.qld.gov.au>

Subject: RE: Cory - ECC/Leave Type

Hi Cory

Thank you for letting me know. As you have not confirmed leave type for the past pay period – I will continue to process as Sick leave / Sick leave no pay

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Friday, 3 July 2026 2:35 PM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Subject: Re: Cory - ECC/Leave Type

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Hey Chloe,

I had a long consultation and the doctor filled out the form.

I sent it off to the Injury Management team today.

Kind regards,

Cory

Get [Outlook for Android](#)

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Thursday, July 2, 2026 2:38:11 pm

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Cc: Cory Shepherd <coryshepherd1@hotmail.com>

Subject: Cory - ECC/Leave Type

Hi Cory,

I just wanted to check in to see how you are doing and how things have been progressing for you. I know you had a scheduled appointment with your doctor yesterday as per text message sent on Monday 29th July.

Did your doctor give you any indication when the required documentation will be completed? Until we receive appropriate medical clearance, including a completed Employee Capacity Certificate (ECC), we are unable to facilitate your return to work, please wait for my advice to when your next returning shift will be. Unless you advise me otherwise, I will process your shifts that you have not worked as Sick Leave – Please confirm by Friday 3rd July before 2:00pm.

Kind regards,
Chloe

Chloe Taylor
[Switchboard Manager](#) | [Local Telecommunications Representative](#) | [Switchboard Services](#)
Logan Hospital | Metro South | Queensland Health
P (07) 2891 9190
E chloe.taylor3@health.qld.gov.au
W www.health.qld.gov.au
A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
T [Call me on Teams](#)







Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this

message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.

**Re: Cory Shepherd_ECC further
information (MSH-INJ-5795)**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

Cc: Notes - IPEC (notes@solv.com.au)

notes@solv.com.au

Sent: Monday 13 July at 16:39

Good afternoon Michelle,

Thank you for your email. Four matters in response.

1. QSuper. I must correct the position stated in your email. I no longer have an active open QSuper Income Protection claim; that claim is now closed. In any event, the supporting medical documentation could only support a partial payment at most: I am not totally incapacitated. I am certified fit for my existing arrangement. There is no income protection payment that could respond to the present situation, in which I am receiving no shifts at all.

My current loss of income arises from the Health Service's decision not to roster me — which your email itself confirms ("we are unable to support a return to work at this time") — and not from my capacity for work. There is accordingly no income support available to me from that source.

2. Pay. My request, made in each of my previous emails — that rostered shifts during this employer-directed period be treated as paid time and not deducted from my leave — has not yet been answered. Given the above, I ask again. If the Health Service's position is that these shifts will be unpaid or deducted from my leave, could you please confirm that in writing, together with the basis for it, so that I can consider my position.

3. Interim alternative duties. Separately, should the Health Service require further time to assess my return to the Switchboard, I note that I also bring prior skills and experience from within Metro South Health, including in the finance department and in scanning/records. I am ready and willing to perform suitable interim duties of that kind, within my certified capacity, as an alternative to remaining off work unpaid while the assessment is completed. This offer is made to my substantive Switchboard position and certified arrangement, to which I remain ready to return. I would welcome a discussion about how my experience can be used to the benefit of the Service in the interim.

4. Dr Ma. My consent for the Health Service to contact Dr Day Hong Ma for clarification, limited to my present work capacity and the recommended adjustments, stands. As I have now been held out of work since 3 July while certified fit, could you please send any questions to Dr Ma promptly, copy me for my records, and advise the timeframe within which my return will be decided once his response is received.

I remain certified fit, ready, and available to work my rostered shifts.

Kind regards,
Cory Shepherd

Get [Outlook for Android](#)

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Monday, 13 July 2026 15:53:26

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Notes - IPEC (notes@solv.com.au)

<notes@solv.com.au>

Subject: RE: Cory Shepherd_ECC further information
(MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email and for providing your response to the further information requested.

I acknowledge that you have advised you do not wish to provide further information regarding the WorkCover matter referenced in your Employee Capability Checklist (ECC) and that you have provided consent for the Health Service to contact your treating practitioner, Dr Day Hong Ma, for clarification regarding your current work capacity and the recommended workplace adjustments.

As mentioned below in the email sent to you from Ms Forrest on 7 July 2026, we are currently unable to safely support or accommodate a Graduated Return to Work Plan that meets your medical requirements. For us to consider your request to return to duties further we require further medical clarification.

Once this information has been received and appropriately reviewed, the Health Service will be in a position to assess whether a return to restricted duties can be facilitated, and we will advise you of the outcome as soon as practicable.

As you currently have an active open QSuper Income Protection claim, any payments that are applicable to you receiving, would be facilitated by them. You should contact your claim manager James Zappia at QSuper and inform them that we are unable to support a return to work at this time.

Should you have any questions in the meantime, please do not hesitate to contact me.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources
Logan and Beaudesert Hospital and Health Service
4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Friday, 10 July 2026 8:35 AM
To: Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: LBH Injury Management
<LBH_injurymanagement@health.qld.gov.au>
Subject: Re: Cory Shepherd_ECC further information

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless**

you recognise the sender and know the content is safe.

Good morning Ms Forrest,

Thank you for your email of 7 July 2026, which I have only now had the opportunity to consider. Apologies for the delay in responding.

I am certified fit to return to work from 3 July 2026 and am ready and available to return. The ECC records the same reduced work arrangement I have performed for approximately the past twelve months. That arrangement is not new; it is now formally documented by my treating practitioner.

I note that the adjustments identified in the ECC appear to be accommodated by existing practice. Your email confirms that Switchboard operators are not expected to manage or resolve consumer complaints, and that current rostering provides more than 10-hour breaks between shifts and is published in advance. Together with my existing shift arrangement, this appears to address the recommendations in the ECC.

I ask that my return to rostered shifts be facilitated, with any remaining clinical clarification obtained in parallel.

Regarding the further information requested:

- **Workplace matters:** These matters are the subject of proceeding WC/2024/227 before the Queensland Industrial Relations Commission. The Health Service is legally represented through Metro South Legal Services, reference K-LM26/729. I will not provide further particulars through this return-to-work process. Any questions about those matters should be directed to Metro South Legal Services.

- **Clinical recommendations:** The recommendations are those of Dr Day Hong Ma. I am not able to interpret or expand on clinical recommendations. If clarification is required, I consent to the Health Service contacting Dr Hong Ma directly, limited only to my present work capacity and the recommended adjustments.
- **Report dated 13 February 2025:** I do not consent to providing this report through this process. It contains sensitive information beyond what is necessary to implement the ECC recommendations. My present fitness for duty is addressed in the ECC and may be clarified by Dr Hong Ma if required.

If the Health Service decides not to roster me pending its assessment, please confirm that these shifts will be treated as paid time and will not be deducted from my leave, as I am certified fit and available to work.

I remain available to commence at my next rostered shift.

Kind regards,
Cory Shepherd

From: Lyndelle Forrest

<Lyndelle.Forrest@health.qld.gov.au>

Sent: Tuesday, July 7, 2026 10:54:35 am

To: coryshepherd1@hotmail.com

<coryshepherd1@hotmail.com>

Cc: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Subject: Cory Shepherd_ECC further information

Good morning Cory

I refer to the Employee Capability Checklist (**ECC**), completed by Dr Day Hong Ma of My Doctor Clinic on 3

July 2026 and returned to the Injury Management Team at 2:30pm that day. It is further noted that due to a section of this document being missing, you were asked to resubmit your ECC for consideration, you sent this on Friday at 3:06pm.

While you have been certified fit to return to work effective from 3 July 2026, this certification is subject to reasonable adjustments being made to your duties and hours of work. As the completed ECC was received late on Friday, 3 July 2026, the Health Service was unable to review Dr Hong Ma's medical recommendations at that time to determine whether the recommended restrictions and adjustments could be reasonably accommodated and whether it was safe for you to return to duty.

I can advise that your ECC has now been reviewed and carefully considered. However, before a final decision can be made regarding your return to the workplace, further information is required. Specifically, it is noted that your treating medical practitioner has advised that there are unresolved workplace matters involving line management. At this stage, the Health Service does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks that may affect your proposed return to work.

The Health Service remains committed to supporting your safe and sustainable return to work, with your health, safety and wellbeing remaining central to that process. Accordingly, further information is required to enable the Health Service to assess any identified risks, determine whether reasonable adjustments can be supported, and ensure that any return to work is planned and managed in accordance with its obligations under work health and safety legislation.

To assist us in expediting consideration of your reasonable adjustments, please provide further clarification and/or reports referenced below, within seven (7) calendar days.

ECC Recommendation	Further information required
<i>Diagnosis of Major Depressive Disorder with anxious distress, a work-related psychological injury with onset 18 June 2024, with symptoms exacerbated on exposure to identified workplace stressors (complaint handling; being held accountable and blamed for failures of others; unpredictable rostering.</i>	The Health Service is seeking to better understand the work-related psychological injury for which you are currently receiving treatment from a psychologist. It is also noted that Dr Day Hong Ma has referenced a report from your treating psychiatrist dated 13 February 2025. To date, the Health Service has not been provided with, nor had the opportunity to review, this report. To enable the Health Service to appropriately consider and address these matters, further information and/or a copy of this report is requested regarding the workplace issues referred to in the ECC and their impact on your capacity for work.
Complaint handling No complaint-handling duties, complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd.	The Health Service understand that the established process for management of complaints is for all patient concerns to be redirection to the Consumer Liaison Services (CLS). There is no expectation for

	Switchboard operators to manage or deal with these matters. Where a consumer raises concerns in relation to a Switchboard employee, these concerns are referred to the Manager, Switchboard or in her absence, CLS. To understand this concern better and adjustments required, please clarify what features of complaint handling are being referenced and you are expected to resolve within this recommendation.
Predictable Rostering practices, minimum 10-hour break between shifts	Rosters are generally published with four (4) weeks' notice, on four-week rotation. Occasionally, in extenuating circumstances these might be delayed. However, all rosters are released with a minimum of two weeks' notice. The Health Service is not aware of breaching this provision and has not received any Workload Concerns to review, can you please clarify the concerns relating to not providing the minimum break

	between rostered shifts? Noting that rostering currently provides more than 10-hour breaks between shifts, however, acknowledge you also signed an 8 hour agreement on 17 June 2020, which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred. Please clarify the specific stressors which may exacerbate symptoms.
Being held accountable and blamed for failures of others	It is unclear what matters you are being held accountable and blamed for based on the failures of others or what these concerns relate to. To-date the Health Service is not aware of any concerns being raised for appropriate management. Further clarity on this would assist in understanding this matter.

Until this information is available and has been appropriately assessed, the Health Service is not in a position to safely facilitate your return to normal duties.

Should you have any further questions or concerns, please do not hesitate to contact me via the details

below.

Kind regards

Lyndelle Forrest
Senior Consultant
Human Resources
Logan and Beaudesert Health Service
[Metro South Health](#) | 

e. Lyndelle.Forrest@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2028](tel:0735422028)
a. Mon - Fri 7:30am - 4:00pm



Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting,

maintaining and ensuring appropriate use of its
computer network.

RE: INFORMATION UPDATE | QSuper
Income Protection Claim: Cory Shepherd
(MSH-INJ-5795) [CLM-317073]

From: LBH Injury Management

LBH.InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

James Zappia james.zappia@art.com.au

Cc: notes@solv.com.au notes@solv.com.au

Sent: Wednesday 15 July at 13:10

Good afternoon Cory,

The only information supplied to QSuper/ART from injury management is the email I cc'd you in on Monday 13/07/2026, which confirmed if the Health Service could support your return or not. This is in line with our process where a QSuper Claimant requests to return to duties.

In relation to conversations, again, these have related to if the Health Service could support your return to duties and that you have a current QSuper Income Protection Claim.

Consent for QSuper (Australian Retirement Trust) to obtain information from Metro South Health was provided when you completed and signed the declaration and authorisation on page 7 of your Income Protection Application. That authorisation permits QSuper to request relevant information from your employer in relation to your claim.

I will endeavour to complete the Request for Medical Information expeditiously; however, I am unable to commit to a timeframe for this.

In relation to your pay request, as we have been provided information that you do hold a current QSuper Income Protection Claim, we are not responsible for your payments whilst on this claim,

that does sit with your QSuper Claim Manager to discuss with you. Where you have received advice from QSuper/ART that your claim has been closed, please provide a copy of this correspondence and/or email from QSuper/ART for our records.

In relation to your request to perform duties in an alternative work area while this matter is being assessed and having regard to the medical information provided by Dr Ma in the Employee Care Centre (ECC) assessment, the Health Service is unable to consider this arrangement at this stage. This is because further medical information is required to understand the nature of the matters identified by Dr Ma and to assess whether they present any psychosocial hazards or risks that may affect your proposed return to work in any capacity. Until this information is received and considered, the Health Service is unable to undertake an appropriate assessment of your request.

I will also clarify that the obligation to provide rehabilitation and suitable duties under the *Workers' Compensation and Rehabilitation Act 2003 (Qld)* applies to workers with accepted workers' compensation claims. It does not extend to employees being managed through Health Management processes or those receiving QSuper (Australian Retirement Trust) Income Protection benefits.

As such, any return to work under a Health Management or QSuper Income Protection arrangement is subject to the employer being able to safely accommodate the medical restrictions within the operational requirements of the workplace and remains at the discretion of the Employer if they are safely able to accommodate this.

Kindest Regards
Michelle Harrison

Injury Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service
4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au | [ams](#)
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 13 July 2026 4:57 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; James

Zappia <james.zappia@art.com.au>

Cc: notes@solv.com.au

Subject: Re: INFORMATION UPDATE | QSuper Income Protection Claim: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Good afternoon Michelle,

I acknowledge receipt of your correspondence to Mr Zappia at QSuper. Four matters in response.

1. Disclosure to QSuper. For my records, could you please confirm what documents and information have been provided to QSuper/ART in relation to me, on what dates, and the authority relied upon for providing my health information to the fund — noting my advice that the income protection claim is closed. Could you also confirm the substance of what was discussed with Mr Zappia by telephone. I will be corresponding with Mr Zappia directly regarding the fund's position.

2. The request to Dr Ma. I note the Request for Medical Information will be forwarded "over the coming days." As I have now been held out of work since 3 July while certified fit, I ask that it be sent within three (3) business days, with a copy provided to me as previously requested, and that you advise the timeframe within which my return to work will be decided once Dr Ma's response is received.

I would also note that 28 August 2026 is Dr Ma's review date for the restrictions — it is not a start date for my capacity to work within them, which is certified now and has been since 3 July. A process paced to late August would leave me without work or income for some eight weeks while certified fit, which I trust is not the Service's intention.

3. Pay. My request — made in each of my previous emails — that rostered shifts during this employer-directed period be treated as paid time and not

deducted from my leave remains unanswered. I renew it. If the Health Service's position is that these shifts will be unpaid or deducted from my leave, please confirm that in writing, together with the basis for it, so that I can consider my position.

4. Interim alternative duties. My offer to perform suitable interim duties within my certified capacity — drawing on my prior Metro South Health experience in the finance department and in scanning/records and to my substantive Switchboard position — also remains open and unanswered. I would welcome a response.

I remain certified fit, ready, and available to work my rostered shifts.

Kind regards,
Cory Shepherd

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Monday, July 13, 2026 4:20:56 pm

To: James Zappia <james.zappia@art.com.au>

Cc: notes@solv.com.au <notes@solv.com.au>; Cory Shepherd <coryshepherd1@hotmail.com>

Subject: INFORMATION UPDATE | QSuper Income Protection Claim: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

Good afternoon James,

Thank you for taking the time to speak with me this afternoon.

As discussed, based on the information currently available, we are unable to facilitate Cory's return to work in accordance with the restrictions outlined in the attached Employee Capabilities Checklist (ECC) completed by his treating practitioner.

To ensure we have sufficient information to make an informed decision, we will be seeking further medical clarification from Dr Day Hong Ma. This will assist us in better understanding the context of the recommended workplace adjustments and whether a restricted return to work can be safely and operationally supported. Once this information has been received, we will undertake a further review of Cory's capacity for work.

I note that Dr Day Hong Ma has nominated a review date of 28 August 2026. Over the coming days, we will forward a Request for Medical Information to Dr Day Hong Ma to obtain further clarification regarding the recommended restrictions, including the reduction in hours from Cory's substantive position, to assist us in determining whether a safe and sustainable return to work can be facilitated.

Please don't hesitate to contact me if you have any questions or require any further information.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) | 

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2744](tel:0735422744)
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management

Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh.InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.

New Payroll Enquiry

From noreply@health.qld.gov.au <noreply@health.qld.gov.au>

Date Wed 2026-07-22 16:23

To Cory Shepherd <Cory.Shepherd@health.qld.gov.au>; Chelsey Meighan
<Chelsey.Meighan@health.qld.gov.au>

Thank you for your payroll enquiry, the following information is supplied for your reference:

Enquiry Status

Log Number: 4438861

Current Status: Pending Investigation

Lodgement Date: 22-Jul-2026

Enquiry Employee Details

Employee PAN: 00388372

Employee Name: Cory Shepherd

Enquiry Details

Enquiry Type: Enquiry

Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH_InjuryManagement@health.qld.gov.au

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest lyndelle.forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Tuesday 28 July at 17:39

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard
management correspondence between 2 and 15 July
2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July

2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General

Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures;

rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.
- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,
Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,
James Zappia james.zappia@art.com.au

Sent: Wednesday 29 July at 16:10

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2744](tel:0735422744)
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @[Solv.com.au](#) email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not

miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -
incorrect application of EB12, Award and QH policies - ECC
3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement
ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015
ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)
ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard
ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)
ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)
ATT13 — QH-POL-231 Special leave (HR Policy C7)
ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl

7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.
- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

- 4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
- 5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
- 6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
- 7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
- 8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any

attachments. If you receive this message in error,
please notify the sender by return email or
telephone and destroy and delete all copies.
Unless stated otherwise, this email represents only
the views of the sender and not the views of the
Queensland Government.

Queensland Health carries out monitoring,
scanning and blocking of emails and attachments
sent from or to addresses within Queensland
Health for the purposes of operating, protecting,
maintaining and ensuring appropriate use of its
computer network.

**Re: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Thursday 30 July at 11:25

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

THE SEQUENCE

I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve

months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

ABOUT THE ATTACHMENTS

To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022:

hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I

have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health

Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34)

and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of

my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through

Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the

refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au | [ams](#)
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Tuesday, 28 July 2026 5:40 PM
To: LBH Injury Management
<LBH_injurymanagement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>
Subject: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services

(AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02

(EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).

- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.
- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while

working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.

**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: Notes - IPEC (notes@solv.com.au)

notes@solv.com.au

Sent: Thursday 30 July at 14:33

Good afternoon Cory,

Thank you for your email.

Please note that we are currently progressing the Request for Medical Information (RFMI) correspondence for delegate approval and will endeavour to expedite this process as quickly as possible. However, we are unable to guarantee when the approved documentation will be available for release.

The usual process is for the approved RFMI documentation to be sent directly to the employee, who can then arrange an appointment with their treating medical practitioner to have the information completed and returned.

If you have any further questions in the meantime, please do not hesitate to contact us.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

- lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Thursday, 30 July 2026 11:26 AM
To: LBH Injury Management
<LBH_injurymanagement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>
Subject: Re: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

THE SEQUENCE

I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

ABOUT THE ATTACHMENTS

To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022: hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the

rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34) and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified

restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award
and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We
acknowledge receipt of your correspondence and the
attached documentation.

As previously advised, the Injury Management Team
requires further information from your treating medical
practitioner before we are able to determine whether
Metro South Health can safely facilitate your return to
duties.

The Employee Capabilities Checklist (ECC) provided has
been reviewed; however, further clarification is required
regarding your functional capacity and the medical
restrictions identified to enable an informed assessment of
whether suitable duties can be accommodated safely and
operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) | 

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)

p. [07 3542 2744](tel:0735422744)

a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.

Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -
incorrect application of EB12, Award and QH policies - ECC
3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction

that I not attend "pending review"; • Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma; • your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073); • your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and • leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement
ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015
ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)
ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard
ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)
ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)
ATT13 — QH-POL-231 Special leave (HR Policy C7)
ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated

modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.

- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.

- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).

- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).

- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.

- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed

off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting,

maintaining and ensuring appropriate use of its
computer network.

**Re: Notice of dispute — clause 1.11 —
attendance, roster, leave and pay from 26
June 2026**

From: Cory Shepherd
coryshepherd1@hotmail.com
To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au,
LBH Injury Management
LBH_InjuryManagement@health.qld.gov.au,
LBH_HR LBH_HR@health.qld.gov.au
Cc: Heath Moran heath.moran@together.org.au,
Emily Petering emily.petering@together.org.au
Sent: Wednesday 5 August at 07:30

Dear Chloe,

Thank you for your response yesterday. My reply is attached. It answers the leave question, sets out my understanding of the position, and asks the questions needed so the correct arrangement can be applied. I have made it a separate document so there is no question about what it is or when it was received.

For payroll purposes, the short position is: I do not consent to my recreation leave being applied, and I have asked that the period be coded as special leave on full pay instead. The detail is in the attachment.

Kind regards,
Cory Shepherd
[0417 400 227](tel:0417400227)

From: Chloe Taylor
<Chloe.Taylor3@health.qld.gov.au>
Sent: Tuesday, August 4, 2026 1:35 PM
To: Cory Shepherd <coryshepherd1@hotmail.com>;
LBH Injury Management
<LBH_InjuryManagement@health.qld.gov.au>;
LBH_HR <LBH_HR@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>

Subject: RE: Notice of dispute — clause 1.11 —
attendance, roster, leave and pay from 26 June 2026

Hi Cory

I am writing in response to your Stage 1 Dispute dated 3 August 2026, in which you disagree with the decision that has prevented you from attending your rostered shifts since 3 July 2026. Your dispute has been lodged in accordance with clause 1.11.2(a) of the *Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12)*.

I wish to acknowledge that clause 1.11.2(a) of EB12 requires that Stage 1 discussions should be conducted within 24 hours. This letter confirms that your dispute has been received and is currently under review. While every effort has been made to progress this matter promptly, the 24-hour timeframe specified in the clause will not be achieved. Upon completion of the review, you will be contacted as soon as practicable to discuss the outcome of your dispute and any appropriate next steps. You will also be provided with an opportunity to raise any questions and discuss the matter further as required.

Please be advised that I am required to process leave on your behalf for the period 20 July 2026 to 2 August 2026. As at today's date, you have a remaining recreation leave balance of **41.21 hours**. Could you please advise whether you would like this recreation leave balance applied to this period? If so, the remaining hours within the fortnight would be recorded as sick leave without pay. I acknowledge that your dispute includes concerns regarding your leave entitlements. However, pending resolution of the dispute, it is necessary that your absence continues to be managed through the appropriate leave arrangements.

As the payroll cut-off for PP04 is today, I would appreciate your response as soon as possible. If I do not receive a response before payroll processing occurs, I will apply your remaining recreation leave balance to ensure you receive the available paid leave entitlement for this period.

Employee Assistance

I understand this may be a difficult time for you. Should you wish to seek further support, the Employee Assistance Program, provided through TELUS Health on [1800 604 640](tel:1800604640), is available free of charge for all employees of Metro South Health. This service is a confidential counselling and support service.

Should you wish to access this service, please contact TELUS Health on [1800 604 640](tel:1800604640) or for Aboriginal and Torres Strait Islander support, please contact [1800 816 152](tel:1800816152).

More information about the support TELUS Health can provide can be found out their website <https://app.one.telushealth.com/> or by downloading the free TELUS Health One app. You can log on to both using the following credentials:

Username: mshhs

Password: mshhs

Kind regards,
Chloe

Chloe Taylor
Switchboard Manager | Local Telecommunications Representative | Switchboard Services
Logan Hospital | Metro South | Queensland Health
P (07) 2891 9190
E chloe.taylor3@health.qld.gov.au
W www.health.qld.gov.au
A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Monday, 3 August 2026 11:38 AM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>; LBH Injury Management
<LBH_InjuryManagement@health.qld.gov.au>; LBH_HR
<LBH_HR@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>

Subject: Notice of dispute — clause 1.11 — attendance,
roster, leave and pay from 26 June 2026

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Dear Chloe,

I attach a notice of dispute under clause 1.11.2(a)
of the certified agreement. It is addressed to you as
my immediate supervisor because the clause
requires that.

I would prefer this resolved at Stage 1 and I am
available to discuss it at any time, including today. I
have asked that a support person and my union
representative be present.

I have raised at part 5 a question about who should
conduct the Stage 1 discussion. I make no
allegation against anyone and will attend
whichever way the Health Service prefers.

Kind regards,
Cory Shepherd
AO3, Switchboard Services, Logan Hospital
[0417 400 227](tel:0417400227)

Disclaimer: This email and any attachments may
contain legally privileged or confidential
information and may be protected by copyright.
You must not use or disclose them other than for
the purposes for which they were supplied. The
privilege or confidentiality attached to this

message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.



Person ID:388372
Distribution Point: Elect
Cory Shepherd
U 12304 3113 Surfers Paradise
SURFERS PARADISE QLD 4217

Messages

Pay Advice

Private and Confidential

Pay Date	12.08.2026	Employee Name	Cory Shepherd
Employer ABN	66329169412	Person ID	388372
Employer Name	QUEENSLAND HEALTH	Sub Position	DHSEA-Admin AO3 (04)
Assignment No(S)	PAN 00388372		

Type	Week 1							Week 2							Hrs / Units	Rate	Amount
	Mon	Tue	Wed	Thu	Fri	Sat	Sun	Mon	Tue	Wed	Thu	Fri	Sat	Sun			
	20/07	21/07	22/07	23/07	24/07	25/07	26/07	27/07	28/07	29/07	30/07	31/07	01/08	02/08			
RDO - No Accrual	0.40	0.40	0.40	0.40	0.40			0.40	0.40	0.40	0.40	0.40			4.00		
NP Sick Leave	7.60	7.60	7.60	7.60	7.60			7.60	7.60	7.60	7.60	7.60			76.00		

Current Fortnight Gross Pay 0.00

Total Gross 0.00

Bank Disbursements

Deductions

Total Deductions 0.00

Net Income 0.00

Employer Super Contributions	Leave Balances (Hrs)	Year To Date
	For your leave balance, please refer to the Leave tab in the my Payslips app	Taxable YTD now on myGov Total Earnings 5,529.89

**RE: ACTION REQUIRED | Request for
Medical Information: Cory Shepherd
(MSH-INJ5795) Specialist attendance 12
August 2026, two matters required before
the report, outstanding wages, interim
return to work**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

LBH_HR LBH_HR@health.qld.gov.au

Cc: LBHS Human Resources Team 1

LBH.HRTeam1@health.qld.gov.au, Info M

[<info@mindandmemoryservice.com.au>](mailto:info@mindandmemoryservice.com.au)

info@mindandmemoryservice.com.au, Emily

Petering emily.petering@together.org.au

Sent: Thursday 13 August at 15:48

Good afternoon Cory,

Thank you for providing your consent for me to speak and correspond with your treating doctor in relation to our request for further medical information; it is greatly appreciated.

I would like to have a quick conversation with you regarding the Employee Authorisation Form (attached), as I think it may be helpful to explain the form in a little more detail and provide you with some clarity about it.

I tried calling you earlier this afternoon, however, the call didn't go through.

Could you please confirm the best phone number to contact you on?

Once I have confirmed your number, I'm happy to give you a call at a time that suits you.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)

p. [07 3542 2744](tel:0735422744)

a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Wednesday, 12 August 2026 6:21 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; LBH_HR

<LBH_HR@health.qld.gov.au>

Cc: LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Info M

<info@mindandmemoryservice.com.au>

<info@mindandmemoryservice.com.au>; Emily Petering

<emily.petering@together.org.au>

Subject: ACTION REQUIRED | Request for Medical Information: Cory Shepherd (MSH-INJ5795)
Specialist attendance 12 August 2026, two matters required before the report, outstanding wages, interim return to work
Importance: High

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Dear Ms Harrison,

I refer to the Request for Medical Information dated 31 July 2026 and the extended timeframe for response. I have copied my union, Together Queensland (Ms Emily Petering), for its awareness and support, and the Mind and Memory Service, so the practice has visibility of how Dr Krishnaiah's requirements are conveyed below and can correct me if I have misstated them.

1. The specialist attendance has occurred

I attended my treating psychiatrist, Dr Krishnaiah of the Mind and Memory Service, on 12 August 2026 to respond to the request.

2. Two matters are required before the report can be provided

As a matter of his clinical reasoning, Dr Krishnaiah requires the following from the Health Service:

(a) The basis of the inquiry, being the incident, change or concern that prompted it. None is identified in the correspondence. The reduced-hours working arrangement the request concerns was formalised by the Health Service itself: it was created in MyHR by my manager on 1 June 2026 and approved by me before its due date of 2 June

2026. The Employee Capability Checklist of 3 July 2026 records that arrangement as a continuation, worked and tolerated without deterioration.

(b) The working arrangement against which capacity is to be assessed. The request describes my employment as permanent full-time (76 hours per fortnight). The operative working arrangement is the one the Health Service formalised on 1 June 2026. The report cannot be prepared without confirmation of which arrangement capacity is to be assessed against.

These requirements may be verified directly with the practice, and I consent to the practice confirming them to the Health Service if contacted. I note that Britney from the Mind and Memory Service has been in direct contact with you and can confirm the practice's view.

Once the two matters above are provided, I will confirm the expected turnaround for the report promptly.

Outstanding wages and hardship

I have received no wages since 13 July 2026, and the fifth week has now begun. I have been held out of the workplace since 26 June 2026, now the sixth week. Over that period my accrued leave has been debited, superannuation contributions have ceased, I am meeting living and financial commitments with no income from any source.

Throughout the same period I have held a current certification of fitness for my role with restrictions (3 July 2026) and have remained available to work under the arrangement the Health Service itself formalised on 1 June 2026. The hardship is therefore not the product of any incapacity or unavailability on my part. It arises from the withholding of work and wages while the basis for the Health Service's own inquiry remains

unidentified, and it compounds with each pay cycle.

While an employer may seek medical clarification, it must be able to identify what prompted the inquiry, assess the information it already holds, and communicate its position. While it considers, it cannot properly leave an employee who is certified fit, and whose working arrangement it formalised itself, without wages or work. At present no basis for either has been identified.

What I ask

(i) That the necessary payroll correction (AVAC) be submitted within 24 hours of this email, with confirmation to me, and my outstanding wages processed as a priority payment rather than held to the next scheduled pay run.

(ii) That pending the Health Service's response on item 2, I be returned to the roster under the existing formalised arrangement of 1 June 2026, consistent with the current certificate of 3 July 2026. If the Health Service's position is that it cannot return me to the roster in the interim, please state that position and the reason for it.

If the AVAC is not confirmed within 24 hours and the matters above are not addressed by Friday the 14th of August 2026, I will consider the position a refusal and take the steps available to me to have the wages and the return to work determined externally. I would much prefer to resolve this administratively.

Kind regards,

Cory Lea Shepherd

0417 400 227 | coryshepherd1@hotmail.com

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.

Re: Follow up on Enquiries

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au

Sent: Friday 28 August at 14:18

Good afternoon Jacqui,

Thank you for your time this week. I have three brief matters to raise.

First, I wish to withdraw the Stage 2 grievance referral. My preference is to work through the outstanding issues directly with you. To be clear, I am withdrawing the referral only. My concerns regarding pay, the leave debited since 3 July 2026, and the adjustments contained in the Employee Capability Checklist dated 3 July 2026 remain unresolved and in dispute. Nevertheless, I am hopeful we can work through these matters constructively.

Second, could you please provide an update on the AVAC following your discussion with Ms Taylor on 24 August?

Third, I attended an appointment with my psychiatrist, Dr Ravikumar Bangalore Krishnaiah, on 27 August 2026. Dr Krishnaiah advised that, despite the authority I provided, Queensland Health has not yet responded to his request for information.

Finally, could someone please contact Dr Krishnaiah's practice so that preparation of the report is not delayed? I would also appreciate being advised of any information being shared with Dr Krishnaiah and the matters on which their opinion is being sought.

And if there are any other updates, I would be glad to have them.

Kind regards,
Cory Shepherd
[0417 400 227](tel:0417400227)
coryshepherd1@hotmail.com

Get [Outlook for Android](#)

From: Jacqui Roberts
<Jacqui.Roberts@health.qld.gov.au>
Sent: Monday, August 24, 2026 12:19:27 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Cc: Emily Petering <emily.petering@together.org.au>;
Terry McQuillan <Terry.McQuillan@health.qld.gov.au>
Subject: RE: Follow up on Enquiries

Good Afternoon Cory

Thank you for sharing this information with me I will review and come back to you with any questions I may have.

Thank you also for confirming the time to finish our catch up, I will put this in our calendars now and you should receive an invitation very soon.

I additionally acknowledge your requests and I am speaking with Chloe this afternoon in relation to applying your leave.

Warm Regards
Jacqui

Jacqui Roberts (she / her)
Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital
[Metro South Health](#) | 
e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)
p. (07) 3542 2384 | 0487 313 927

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Monday, 24 August 2026 11:13 AM
To: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>
Cc: Emily Petering <emily.petering@together.org.au>;
Terry McQuillan <Terry.McQuillan@health.qld.gov.au>
Subject: Re: Follow up on Enquiries

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Good morning,

I apologise for Friday and for the state I was in.

I can confirm I am available on Tuesday, 25 August 2026, between 3:00 pm and 5:00 pm.

For ease of reference, I attach:

- the QSuper letter
- the communication I believe prompted the ECC
- my movement details
- my agreement to the sharing of medical information and correspondence with my treating psychiatrist

In the circumstances, I ask to be advised, in advance, of what information is being shared with my treating psychiatrist, what questions are being asked of them, and what is proposed to be included in any correspondence with them.

In the interim, while Metro South Health seeks clarification regarding my medical information, I respectfully request that the appropriate leave be applied and that an AVAC be submitted on my

behalf to facilitate the processing of my entitlements.

Kind regards,
Cory

Get [Outlook for Android](#)

From: Jacqui Roberts
<Jacqui.Roberts@health.qld.gov.au>
Sent: Monday, August 24, 2026 7:44:58 am
To: Cory Shepherd <coryshepherd1@hotmail.com>
Cc: Emily Petering <emily.petering@together.org.au>;
Terry McQuillan <Terry.McQuillan@health.qld.gov.au>
Subject: RE: Follow up on Enquiries

Good Morning Cory

Sorry technology let us down on Friday to finalise our catch up.

I have the following availability for us to conclude our discussions, can you advise which of these times would work for you, Emily has confirmed she is also available to attend at these times too

- Tuesday 25 August 2026 – Between 3pm and 5pm, or
- Friday 28 August 2026 – Between 9am and 11am

If you let me know when you can and I will book it in for us all.

Warm Regards
Jacqui

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Tuesday, 18 August 2026 5:22 PM
To: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>
Subject: Re: Follow up on Enquiries

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Hey Jacqui,

I can confirm that 14:30 to 16:30 works for me.

Thanks for organising this, and I look forward to catching up with you and Emily on Friday.

Kind regards,

Cory

Get [Outlook for Android](#)

From: Jacqui Roberts
<Jacqui.Roberts@health.qld.gov.au>
Sent: Tuesday, August 18, 2026 3:22:54 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Cc: Emily Petering <emily.petering@together.org.au>;
LBHS Human Resources Team 1
<LBH.HRTeam1@health.qld.gov.au>; Cory Shepherd
<Cory.Shepherd@health.qld.gov.au>
Subject: RE: Follow up on Enquiries

Good Afternoon Cory

Thank you for coming back to me and also listing all of your points you would like to discuss.

Emily has reached out this afternoon to advise she is no longer available on Friday morning – would you be free on Friday afternoon?

Thank you
Jacqui

Jacqui Roberts (she / her)
Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital
[Metro South Health](#) | 

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)
p. (07) 3542 2384 | 0487 313 927

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal and Mununjali peoples – and to Elders, past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Tuesday, 18 August 2026 11:33 AM
To: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>
Cc: Emily Petering <emily.petering@together.org.au>;
LBHS Human Resources Team 1
<LBH.HRTeam1@health.qld.gov.au>; Cory Shepherd
<Cory.Shepherd@health.qld.gov.au>
Subject: Re: Follow up on Enquiries

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Dear Jacqui,

Thank you for your email and for including Emily Petering.

I confirm the meeting on Friday, 21 August, from 9:30 am to 11:30 am via Teams, with Emily attending. To ensure the time is used effectively, I expect to discuss the following matters:

- Restoration of my pay, including my request of 28 July 2026 regarding paid special leave.

- Recrediting of all leave debited since 3 July 2026.
- Implementation of the adjustments certified in the Employee Capability Checklist dated 3 July 2026, and my return to work.
- My requests of 3 August 2026, including the flexible working request under the Agreement, for which the decision period ends on 24 August 2026.
- The Stage 2 grievance referral.
-

If there are additional matters you intend to raise, I would appreciate receiving notice of them before Friday.

Medical position – correction

For accuracy, fitness to return to work with adjustments is not my personal view. It is the certification provided by Dr Ma in the Employee Capability Checklist dated 3 July 2026, completed on the Health Service’s own form and at the Health Service’s request.

In addition, a specialist psychiatric report responding to the Health Service’s request for medical information dated 31 July 2026 has been commissioned and will be provided once completed.

•

Leave

To maintain my income in the interim, I do not object to the Health Service processing my available sick leave and, if necessary, annual leave. However, I wish to be clear about the basis on which this occurs:

-
- Any leave processing is being undertaken by the Health Service to ensure continuity of payment and does not represent an election by me.

- My position remains as set out on 3 August 2026: this absence results from the Health Service's decision and should not be debited against my leave balances.
- This arrangement is interim only and subject to the specialist psychiatric report. Once that report is provided, I expect the certified adjustments to be implemented, my return to work to proceed, and my pay to be restored.
- All leave debited since 3 July 2026 should be recredited.
-

Please also review my payslip dated 12 August, which recorded 76 hours as sick leave without pay despite the balances identified in your email. If an AVAC is required to process the leave and correct the payslip, please arrange for this to be submitted promptly and advise if anything further is required from me.

-

QSuper

For accuracy, benefit payments ceased earlier this year following my request to the fund when I returned to work. I have not received any income protection payments since 31 May 2026 and am not currently in receipt of such payments.

The administration of the claim is a matter I am addressing directly with the fund's Claims Manager. Superannuation contributions continue to be reported through payroll in the ordinary course.

-

Thank you for arranging this meeting. I look forward to speaking with you on Friday.

Kind regards,
Cory Lea Shepherd

From: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>

Sent: Tuesday, August 18, 2026 10:47:48 am

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Emily Petering <emily.petering@together.org.au>;

LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Cory Shepherd

<Cory.Shepherd@health.qld.gov.au>

Subject: FW: Follow up on Enquiries

Good Morning Cory

My apologies – I have just noticed I had sent this email to your health account only and did not include your personal, forwarding for your information in case you have not accessed your health email account.

If you could get back to me as soon as possible regarding my meeting suggestion, it would be very much appreciated

Warm Regards

Jacqui

Jacqui Roberts (she / her)

Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital

[Metro South Health](#) | 

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)

p. [\(07\) 3542 2384](tel:(07)35422384) | [0487 313 927](tel:0487313927)

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal and Mununjali peoples – and to Elders, past and present.

From: Jacqui Roberts

Sent: Thursday, 13 August 2026 4:46 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Cc: LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Emily Petering

<emily.petering@together.org.au>

Subject: Follow up on Enquiries

Good Afternoon Cory

I hope this email finds you well, I wanted to reach out to you in relation to your emails and enquiries you have made over the past couple of weeks and would welcome time to catch up with you to discuss these further to assist in resolving the concerns you have raised.

My availability is a little limited at the moment as I am on leave for the next few days but I do have time on Friday 21 August 2026 and would like to book in a minimum of about 2 hours with you to really discuss and understand your matters further. The times I have available are

- 09:30 to 11:30, or
- 14:30 to 16:30

I propose we can do this via teams and also include Emily Petering too, you will see I have included her in my email.

If you could confirm which of these times would work for you **by close of business on Wednesday 19 August**, that would be really helpful.

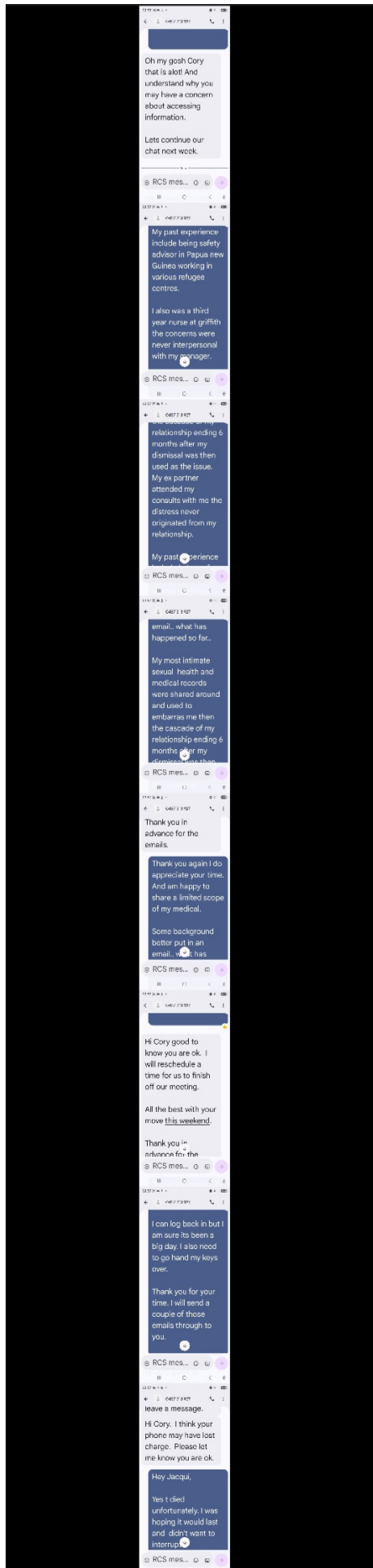
Whilst I understand your belief you are fit to return to the workplace; the medical evidence provided to date does not support that. Therefore, this leave would ordinarily be processed as sick leave and if no sick leave balances are available, it will automatically be processed as sick leave no pay. However, to avoid not being paid, with your approval we can apply other leave entitlements, please see below your current balances. If you could confirm what leave you wish to be applied from these balances, and we can action accordingly. You do have sick leave balances, so they should automatically be applied, and this is something we can follow up on for you.

I am aware you have a current open Qsuper claim, and we may need to advise Qsuper of any income you are in receipt of in relation to that claim.

the views of the sender and not the views of the
Queensland Government.

Queensland Health carries out monitoring,
scanning and blocking of emails and attachments
sent from or to addresses within Queensland
Health for the purposes of operating, protecting,
maintaining and ensuring appropriate use of its
computer network.

TAB 13 — SMS thread, Roberts ↔ Shepherd, August 2026 (screenshot as captured)



Meeting - Cory Shepherd

From: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

Terry McQuillan

Terry.McQuillan@health.qld.gov.au, Emily Petering

emily.petering@together.org.au

Sent: Tuesday 25 August at 15:08

**Update 25/08/2026 – For Internal information only –
adding in meeting room, not relevant for Emily or Cory**

Good Afternoon Cory

As arranged, here is the link to join the meeting to
continue our discussions from Friday.

Warm Regards

Jacqui

Microsoft Teams meeting

Join:

[https://teams.microsoft.com/meet/411](https://teams.microsoft.com/meet/411575551338689?p=zBTEMPwfHn13HT1TLE)

[575551338689?](https://teams.microsoft.com/meet/411575551338689?p=zBTEMPwfHn13HT1TLE)

[p=zBTEMPwfHn13HT1TLE](https://teams.microsoft.com/meet/411575551338689?p=zBTEMPwfHn13HT1TLE)

Meeting ID: 411 575 551 338 689

Passcode: dF6EL6MP

[Need help?](#) | [System reference](#)

Dial in by phone

[+61 7 3186 0111,,735268758#](tel:+61731860111735268758) Australia, Brisbane

[Find a local number](#)

Phone conference ID: [735 268 758#](tel:+61731860111735268758)

Join on a video conferencing device

Tenant key: qldhealth@m.webex.com

Video ID: 137 555 063 4

[More info](#)

For organizers: [Meeting options](#) | [Reset dial-in PIN](#)



Unauthorised access to or use of this system is strictly prohibited. Authorised users must not allow others to use their passwords or access tokens. Inappropriate use may result in disciplinary action. Questions concerning usage policy should be directed to a user’s supervisor in the first instance. Queensland Health monitors the use of its systems, including internet access and email. By accessing and using Queensland Health’s systems, you consent to such monitoring activity by Queensland Health in respect of your use of those systems. Please use Queensland Health’s systems responsibly.

[Org help](#) | [Privacy and security](#)

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose,

retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.

**Re: Cory Shepherd (388372) —
application for long service leave and
annual leave, and request for AVAC to be
processed today**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au

Cc: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au,

emily.petering@together.org.au

emily.petering@together.org.au

Sent: Monday 31 August at 16:06

Dear Chloe,

Thank you for actioning this today and for the reference number — I appreciate the quick turnaround.

Four short points so the record is accurate.

1. Leave type. To confirm my application: I did not apply for sick leave — I am not unfit for work, and the Employee Capability Checklist of 3 July 2026 certifies me fit with restrictions. My application was for long service leave and, on its exhaustion, annual leave. Could you please confirm exactly which leave types and how many hours were applied for 17–30 August 2026 under PRN 249 863 66. If any personal or sick leave has been debited, could it please be re-credited and replaced in accordance with my application.

2. Hours. Could the leave please be applied at my substantive hours of 76 per fortnight rather than at 0.5 FTE. The reduced-hours arrangements this year were temporary changes to working conditions: the most recent, approved on 9 June 2026, had an effective period of 25 May 2026 to 28 June 2026 and was not extended.

No variation has been in force since 28 June 2026, and my pay advice of 12 August 2026 itself records my absence at 76.00 hours for the fortnight. If a different arrangement has been recorded for any period after 28 June 2026, could you please let me know what it is and the document it is based on.

3. The period from 13 July. My application was from 13 July 2026, being the date from which I have received no wages. I understand the long service leave question is with Payroll — thank you for raising it promptly. Once it is resolved, could the leave please be applied back to 13 July 2026 as an adjustment, and the "NP Sick Leave" recorded for the earlier fortnights corrected at the same time.

4. Long service leave — what may assist Payroll. My employment commenced on 25 March 2019, and my reinstatement in February 2025 took effect from 20 September 2024 with continuity of service preserved. I therefore reached seven years' continuous service on 25 March 2026 and am entitled to take long service leave under clause 22(c) of the Award. If the system is showing a later continuous service date, it may not have been corrected following my reinstatement — that would explain the error, and Payroll can verify it against my record. I have separately written to Payroll today about my service record and leave balances. Could you also confirm that the payment for 17–30 August will be processed urgently?

For completeness, everything actioned today, and any adjustments, remain on the basis set out at paragraph 4 of my email this morning — under protest and subject to re-credit.

Thanks again, Chloe.

Kind regards,
Cory Shepherd

Employee number 388372

0417 400 227

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Monday, August 31, 2026 3:34:24 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>

Subject: RE: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Good afternoon Cory,

Thank you for your email. I have applied your S/L and A/L in lieu of S/L for your absence from the workplace through an AVAC as requested, for the past fortnight 17th August 2026 – 30th August 2026. As you are on reduced roster 0.5 FTE, I have applied the leave as per reduced shifts - PRN 249 863 66

About your long service leave from the 13th July 2026 - the system is currently not allowing me to apply LSL, so I have made enquiries with Payroll to understand why this may be and I will get back you as soon as I have an update.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 31 August 2026 10:59 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;
emily.petering@together.org.au

Subject: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is
safe.**

Dear Chloe,

I am writing to apply for leave and to ask that the AVAC be processed today. I have copied Jacqui Roberts.

1. MY POSITION

I have not been working since 3 July 2026. I have received no wages since 13 July 2026.

On 3 July 2026 Dr Ma completed the Employee Capability Checklist on Metro South's own form. It certifies me fit with restrictions and records that usual switchboard operational duties remain suitable. I have remained ready and available to work within those restrictions throughout, and I remain so now.

My pay advice of 12 August 2026, for the fortnight 20 July to 2 August 2026, records me as on "NP Sick Leave" for 76 hours with gross pay of \$0.00. I did not apply for sick leave and I am not unfit for work.

2. WHAT I APPLY FOR

In the interim, and so that my entitlements can be processed, I apply as follows:

- Long service leave, on full pay, from 13 July 2026 being the date from which I have received no wages through to the current date and continuing; and
- when my long service leave entitlement is exhausted, annual leave, to commence immediately from that point and to continue until further notice.

To the extent that any absence over that period has already been recorded as unpaid sick leave, I ask that the coding be corrected and replaced with the leave applied for above.

I confirm I am not applying for personal leave, as I am not unfit for work.

3. THE AVAC

I ask that the AVAC be submitted and processed today, and that I be given the reference number once it has been done, together with confirmation of the pay run in which the payment will be made.

I ask this because of the financial position set out below, and because the cut-off for the next available pay run is imminent.

4. THIS APPLICATION IS MADE UNDER PROTEST, AND SUBJECT TO RE-CREDIT

This application is made under protest and without prejudice to my position, and on the following express basis:

- I remain ready, willing and able to work within the restrictions recorded in the Employee Capability Checklist of 3 July 2026, and I have been since 3 July 2026;
- I apply for this leave only because I have received no wages since 13 July 2026 and no payment has been made to me in the interim;

- my concerns regarding pay, the leave debited since 3 July 2026, and the adjustments recorded in the Employee Capability Checklist of 3 July 2026 remain unresolved and in dispute, and nothing in this application resolves, waives or narrows any of them; and
- I ask that the leave taken under this application be re-credited to me in the event that wages are paid, or found payable, for any part of the same period, or in the event that the recording of my absence over that period is later corrected or determined otherwise.

I make this application on the understanding that it does not prevent that outcome, and I reserve my position in full.

5. FINANCIAL HARDSHIP

I want to be plain about why I am asking for this to be done today rather than in the ordinary course.

I have had no income since 13 July 2026. As a consequence I have had to give up my accommodation. I am not raising this to make a point; I am raising it because it is the reason for the urgency, and because I would rather tell you than have it become apparent later.

If there is any mechanism available for an urgent or off-cycle payment in cases of financial hardship, I would be grateful if you could tell me and I will apply for it.

6. IF ANY PART OF THIS CANNOT BE ACTIONED

If any part of this application cannot be actioned today, could you please let me know today which part, and why, and when it will be actioned. A short note is entirely sufficient.

Thank you for your assistance.

Kind regards,
Cory Shepherd
Employee number 388372
0417 400 227
coryshepherd1@hotmail.com

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies. Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

Queensland Health carries out monitoring, scanning and blocking of emails and attachments sent from or to addresses within Queensland Health for the purposes of operating, protecting, maintaining and ensuring appropriate use of its computer network.

